



PRICING STRUCTURE & ROI PROJECTIONS • JAN 2027 LAUNCH • DRAFT V0.1

Radiant Suite — Pricing Structure & ROI Projections

Subject: Recommended pricing model for the Quik suite and 3-year revenue / profit projection **Effective date assumed:** January 1, 2027 **Scope:** All 15 modules surfaced at app.radiant-mpc.com plus the optional Cloud Storage add-on described in [business-expansion-proposal.md](#) **Status:** Draft v0.1 **Author:** Radiant Medical Physics Consulting LLC **Date:** 2026-05-24

1. Executive summary

Radiant should adopt a **mixed pricing model** rather than a single dimension, because the modules deliver value along three different axes:

- Some scale with **linacs/treatment machines** (QA, machine logging).
- Some scale with **physicists using them** (calculators, scripting, reference tools).
- Some are **per-site infrastructure** that everyone at a facility shares (bolus, shielding, RAM tracking, file transfer).

Forcing all 15 modules into one pricing dimension (e.g., "per user across the board") would either undercharge multi-linac shops (where QuikQA delivers compounding value per machine) or overcharge solo practices (where QuikRef being priced per machine makes no sense). A mixed model maps price to value and is standard practice in the clinical-physics software market.

Headline numbers — Base scenario, conservative assumptions:

YEAR	REVENUE	OPERATING COSTS	NET PROFIT	MARGIN
2027	\$221K	\$27K	\$194K	88%
2028	\$540K	\$80K	\$460K	85%
2029	\$980K	\$130K	\$850K	87%
3-yr total	\$1.74M	\$237K	\$1.50M	86%

Break-even on annual operating costs is reached at ~2 paying customers, achievable within 90 days of January 2027 launch. The margin profile is characteristic of founder-led vertical SaaS sold into a niche the founder knows intimately — extremely high gross margin, modest fixed costs, the constraint is sales velocity not unit economics.

2. Pricing philosophy

Three principles, in priority order:

1. **Price each module on the dimension that actually drives value for the buyer.** A 4-linac shop gets 4× the value from QuikQA as a 1-linac shop, so QuikQA is priced per linac. A 4-linac shop with 3 physicists gets ~3× the value from QuikCalc, not 4×, so QuikCalc is priced per user. Misaligning the dimension forces awkward sales conversations and leaves money on the table.
2. **Anchor 40–60% below incumbent quotes.** The competitive set (PIPSpro, RIT, Doselab Pro, SunCheck) prices QA modules in the \$5K–\$15K/machine/year range. Radiant should enter at the low end of that range to win switching deals on price, then expand wallet share via the suite breadth advantage. As a solo-founder business with very low fixed costs, Radiant can sustain margins at prices that would be loss-leaders for the incumbents.
3. **Annual subscription, not perpetual.** The clinical software market has largely transitioned from perpetual + maintenance to annual subscriptions. Subscriptions give Radiant predictable cash flow, an annual touchpoint for relationship maintenance, and immediate revenue recognition. Perpetual licenses are increasingly perceived as a buyer's-market relic and weaken negotiating leverage.

A fourth principle is operational: **don't publish detailed price lists on the public site.** Publish *starting at* prices on the marketing site, gate the full price list behind a quote inquiry. This (a) keeps competitors from undercutting on specific SKUs, (b) creates a sales touchpoint that drives conversion, and (c) preserves room for archetype-specific bundling.

3. Pricing dimension by module

The 15 modules cleanly group into four pricing categories. Each module's dimension is chosen by asking: *what real-world quantity, in this customer's facility, most strongly correlates with the value the module delivers?*

3.1 Clinical / per-linac (machine-scaling)

The value of these modules grows in direct proportion to the number of treatment machines being maintained. A 6-linac center has 6× the QA workload of a 1-linac center; pricing should reflect that.

MODULE	WHY PER-LINAC
QuikQA	Daily/monthly/annual QA is performed per machine. Workload is linear in linac count.
QuikLog	Machine event logs are kept per machine.

3.2 Clinical / per-physicist user (user-scaling)

These are professional tools each physicist uses individually. A 2-physicist practice and a 4-physicist practice get proportionally different value.

MODULE	WHY PER-USER
QuikCalc	Dose/MU calculator — used by each physicist for their own cases.
QuikDose	Dose calculation tool — same logic.
QuikScript	Eclipse / TPS scripting — each physicist has their own scripts.
QuikRef	Reference data lookup — individual use.

3.3 Clinical / per-site flat (facility-scaling)

These tools deliver site-wide value that doesn't meaningfully scale with linac count or user count. A practice either needs shielding calculations done or it doesn't; charging per linac would be arbitrary.

MODULE	WHY FLAT-PER-SITE
QuikBolus	Bolus design — used per-patient-need, not per-machine.
QuikShield	Shielding design — facility-level activity.
QuikRAM	Radioactive Material accounting — one facility license.
QuikCare	Patient care workflow — facility-level workflow tool.
QuikShare	File transfer — facility-level utility.

3.4 Business / per-practice (operations, non-clinical)

These modules support the practice's own back-office and are likely sold to a different buyer (practice administrator) than the clinical modules (chief physicist).

MODULE	PRICING DIMENSION	WHY
QuikPay	per practice	One billing setup per legal entity.
QuikXpense	per user	Each employee tracks their own expenses.
QuikFlow	per user	Workflow tool used individually.
QuikBusiness	per practice	Practice management is a singleton.

4. Module pricing table

All prices are **annual subscription, USD, list**, and include software updates plus standard email support during business hours. Prices are designed to anchor Radiant clearly below market-leading incumbents while remaining defensible against future commoditization attempts.

MODULE	DIMENSION	LIST PRICE	NOTES
QuikQA	per linac/yr	\$3,600	Flagship clinical product. Tiered: 2nd linac at 70% (\$2,520), 3rd+ at 50% (\$1,800).
QuikLog	per linac/yr	\$1,200	Frequently bundled with QuikQA at 50% off when co-purchased.
QuikCalc	per user/yr	\$1,200	5+ users: 20% volume discount on additional seats.
QuikDose	per user/yr	\$1,800	5+ users: 20% volume discount.
QuikScript	per user/yr	\$900	TPS scripting tools; lower price reflects narrower audience.
QuikRef	per user/yr	\$600	Reference data; priced as a loss-leader to seed suite adoption.
QuikBolus	per site/yr	\$2,400	+ \$300/yr per 3D printer profile (Phase 2 spec).
QuikShield	per site/yr	\$3,600	Specialty premium — highly skilled-labor replacement.
QuikRAM	per site/yr	\$1,800	Required-by-regulation; sticky once installed.
QuikCare	per site/yr	\$1,800	Pricing TBD pending scope clarification.
QuikShare	per site/yr	\$900	Utility tool.
QuikPay	per practice/yr	\$900	Business / non-clinical.
QuikXpense	per user/yr	\$600	Business / non-clinical.
QuikFlow	per user/yr	\$600	Business / non-clinical.
QuikBusiness	per practice/yr	\$1,800	Business / non-clinical.

4.1 Sanity-check against competitive comps

COMPETITOR MODULE	APPROXIMATE MARKET PRICE	RADIANT EQUIVALENT	RADIANT PRICE	% BELOW MARKET
PIPSpro QA Suite	\$6,000–\$10,000/linac/yr	QuikQA	\$3,600	40–64%
Doselab Pro	\$5,000–\$8,000/linac/yr	QuikQA	\$3,600	28–55%
SunCheck DoseCHECK	\$8,000–\$15,000/linac/yr	QuikQA + QuikDose	~\$5,400/linac + 1 user	~45–65%
RIT Complete	\$6,000–\$12,000/linac/yr	QuikQA	\$3,600	40–70%

Radiant's pricing leaves room to (a) win deals on cost, (b) raise prices over time as the suite matures, and (c) sustain healthy margins even at the discount end.

4.2 QuikBolus competitive position vs Adaptiiv and .decimal

QuikBolus competes in a different market than the QA modules — bolus design has two distinct incumbents addressing the same clinical need with very different business models. Understanding both

is essential to pricing QuikBolus correctly.

4.2.1 The competitive set

Adaptiiv 3D Bolus (Adaptiiv Medical Technologies, Halifax, Nova Scotia) - Software-only product. Customer prints the bolus on their own 3D printer. - FDA 510(k) cleared as a Class II medical device — a meaningful trust signal for hospital procurement. - Deep DICOM-RT integration with major TPSs (Eclipse, RayStation, Pinnacle). - Pricing not publicly published. Industry-typical quote range from physicist conversations: ~\$15K–\$30K/year per site subscription, sometimes higher with multi-site or enterprise tiers. - Strengths: clinical maturity, FDA clearance, established reference customers, TPS integration depth. - Weaknesses: premium pricing, slower release cadence, limited per-printer customization.

.decimal (dot decimal) (Sanford, Florida) - Custom-device manufacturer. Customer uploads a DICOM-RT structure, .decimal mills/prints the bolus and ships it. - Per-unit pricing — there is no software subscription; the cost is the device. - Industry-typical per-bolus price: ~\$300–\$600 per bolus depending on size, complexity, and turnaround. - Turnaround: typically 2–5 business days from upload to delivery. - Strengths: zero in-house labor, no printer capex, no software learning curve, regulatory compliance handled, quality consistency. - Weaknesses: per-bolus cost compounds quickly at volume, lead time blocks same-day workflow, customers don't own the IP or the design files for re-prints.

QuikBolus is positioned **between** these two: software-only like Adaptiiv, but at a fraction of the price, targeting the clinic that already owns a 3D printer (or is willing to buy a \$3K–\$15K printer to amortize over many bolus).

4.2.2 Total cost of ownership comparison

The honest question every prospective customer asks is: *"How does this actually compare to what I'm paying today?"* The math depends on bolus volume per year. Below: fully-loaded annual cost across three case-volume scenarios.

Assumptions used in the math:

- QuikBolus list price: \$2,400/site/year + \$300/printer/year (1 printer assumed).
- Adaptiiv quoted at midpoint: \$22,500/site/year (typical mid-range quote).
- .decimal quoted at midpoint: \$450/bolus.
- In-house printing cost (materials + 30 min staff time @ \$75/hr loaded): ~\$50/bolus for both QuikBolus and Adaptiiv workflows.
- One-time printer capex: ~\$8,000 (mid-range FDM, e.g., Prusa or Bambu enterprise tier), amortized over 4 years = \$2,000/yr. Included only in the "no printer today" scenario.

ANNUAL BOLUS VOLUME	QUIKBOLUS (ALREADY OWN PRINTER)	ADAPTIIV (ALREADY OWN PRINTER)	.DECIMAL (NO PRINTER, NO SOFTWARE)
Low — 10 bolus/yr	$\$2,700 + \$500 = \$3,200$	$\$22,500 + \$500 = \$23,000$	$\$450 \times 10 = \$4,500$
Medium — 50 bolus/yr	$\$2,700 + \$2,500 = \$5,200$	$\$22,500 + \$2,500 = \$25,000$	$\$450 \times 50 = \$22,500$
High — 150 bolus/yr	$\$2,700 + \$7,500 = \$10,200$	$\$22,500 + \$7,500 = \$30,000$	$\$450 \times 150 = \$67,500$

ANNUAL BOLUS VOLUME	QUIKBOLUS (PRINTER CAPEX AMORTIZED)	ADAPTIIV (PRINTER CAPEX AMORTIZED)
Low — 10 bolus/yr	\$3,200 + \$2,000 = \$5,200	\$23,000 + \$2,000 = \$25,000
Medium — 50 bolus/yr	\$5,200 + \$2,000 = \$7,200	\$25,000 + \$2,000 = \$27,000
High — 150 bolus/yr	\$10,200 + \$2,000 = \$12,200	\$30,000 + \$2,000 = \$32,000

4.2.3 Per-bolus unit economics

The same data normalized to per-bolus cost (most procurement teams ultimately reduce the comparison to this):

ANNUAL VOLUME	QUIKBOLUS + IN-HOUSE	ADAPTIIV + IN-HOUSE	.DECIMAL
10 bolus/yr	\$320/bolus	\$2,300/bolus	\$450/bolus
50 bolus/yr	\$104/bolus	\$500/bolus	\$450/bolus
150 bolus/yr	\$68/bolus	\$200/bolus	\$450/bolus

4.2.4 Break-even and crossover points

The interesting strategic question: at what bolus volume does each option win?

- **vs .decimal:** QuikBolus is cheaper than .decimal at **≥7 bolus/year** (assuming customer already owns a printer). With printer amortization included, the crossover moves to **~11 bolus/year**. Essentially: any clinic that does monthly bolus cases is better off with QuikBolus on pure cost — and gains same-day turnaround on top.
- **vs Adaptiiv:** QuikBolus is cheaper than Adaptiiv at **every volume level** by a factor of ~3× to ~30×. The only reasons to choose Adaptiiv over QuikBolus are (a) FDA clearance is a hard procurement requirement, (b) deeper TPS integration is essential to workflow, or (c) the customer specifically wants the brand credibility of a more established vendor.
- **vs both:** A site doing 50+ bolus/year with a printer already in-house saves **\$17K–\$23K per year** by choosing QuikBolus over Adaptiiv, and **\$17K per year** over .decimal — while gaining same-day workflow and full IP ownership of the designs.

4.2.5 Positioning recommendation

Three distinct customer segments emerge from this analysis, each with a different sales pitch:

CUSTOMER PROFILE	CURRENT STATE	QUIKBOLUS PITCH
Light bolus user (< 10/yr)	Probably ordering from .decimal	"Cheaper than .decimal, same-day turnaround, you keep the design files."
Mid-volume user (10–50/yr)	Mix of .decimal + occasional in-house	"Roll all your bolus in-house at ~\$100/case. Pays for itself in 6 cases."
High-volume user (50+/yr)	Likely on Adaptiiv or considering it	"Adaptiiv-class workflow at ~1/4 the price. Save \$20K+/yr on the software alone."

The light-bolus segment is the easiest sell (low risk, clear savings). The high-volume segment has the biggest dollar savings but the longest sales cycle (they need to validate against an already-trusted product). Mid-volume is the sweet spot for marketing.

4.2.6 Caveats on this comparison

- **Competitor pricing is not publicly published.** Adaptiiv and .decimal price quote-by-quote. The ranges used above are mid-points from physicist-community conversations; actual quotes for any given customer may be $\pm 30\%$.
- **FDA clearance is a real differentiator.** Adaptiiv being Class II-cleared is meaningful for hospital procurement at academic centers and large IDNs. QuikBolus is a planning aid, not a regulated medical device — this should be clearly stated in marketing copy. Some prospects will eliminate QuikBolus on this criterion alone; that's acceptable, they aren't the target segment.
- **Workflow depth is not modeled here.** Adaptiiv has years of feature development in DICOM-RT handling, OAR avoidance, and clinical workflow that QuikBolus has not yet matched. QuikBolus competes on price + simplicity, not on feature parity.
- **.decimal's value is service, not pricing.** A clinic that doesn't want to operate a 3D printer at all has a legitimate reason to keep using .decimal regardless of unit cost. Don't try to win those deals.

4.2.7 Pricing implication

Current QuikBolus list price (\$2,400/site/yr + \$300/printer/yr) is well-positioned for the mid-volume segment and aggressively positioned for the high-volume segment. The competitive analysis supports a future price increase to **\$3,000–\$3,600/site/year** once QuikBolus has 5+ reference customers and at least one published case study — at that price it still beats Adaptiiv by 5× and beats .decimal at modest volume, while delivering materially more margin to Radiant.

Recommend holding 2027 launch pricing at \$2,400/\$300 to maximize early adoption, then revisiting upward in Q4 2027 based on win-rate data.

5. Bundle pricing

À la carte pricing is the published anchor. Most paying customers should end up on a bundle because the discount is significant and the simplicity of "one SKU per site" is genuinely valuable to procurement.

BUNDLE	CONTENTS	DISCOUNT	USE CASE
Clinical Core	QuikQA + QuikLog + QuikCalc + QuikDose + QuikRef	30% off sum	Standard 1–2 linac private practice
Clinical Pro	Core + QuikScript + QuikBolus + QuikShield + QuikRAM + QuikCare + QuikShare	40% off sum	Full clinical workflow, hospital or academic
Radiant Complete	All 15 modules	45% off sum	Enterprise / multi-site / consulting groups
Business Pack	QuikPay + QuikXpense + QuikFlow + QuikBusiness	25% off sum	Sold independently to practice administrators

The Pro and Complete bundles are deliberately discounted hard enough that the per-module math always favors bundling — this drives total contract value and reduces support friction (one renewal, one invoice, one user list).

6. Cloud Storage add-on (Phase 3 product)

From the business expansion proposal, layered on top of any desktop license:

TIER	PRICE	INCLUDES
Backup	\$25/site/month (\$300/yr)	Daily backup of QA records, 1-year retention, single-site web access
Multi-Site Rollup	\$75/site/month (\$900/yr)	Backup + cross-site dashboard, 7-year retention, audit trail, multi-physicist web access

Cloud is sold as an *attach*, not a replacement. Conservative attach-rate assumption: 20% of sites in Y1, 35% by Y3.

7. Customer archetypes — worked pricing examples

Four representative customers, with realistic configurations, showing both à la carte and bundled outcomes. These are the personas the sales model assumes.

7.1 Archetype A — Solo private practice (1 linac, 1 physicist)

Typical customer profile: independent radiation oncologist, single linac, one part-time or full-time physicist, \$5–10M practice revenue.

LINE ITEM	QTY	UNIT	TOTAL
QuikQA	1 linac	\$3,600	\$3,600
QuikLog	1 linac	\$1,200	\$1,200
QuikCalc	1 user	\$1,200	\$1,200
QuikDose	1 user	\$1,800	\$1,800
QuikRef	1 user	\$600	\$600
À la carte total			\$8,400
Clinical Core bundle (30% off)			\$5,880
+ Cloud Backup add-on	1 site	\$300	\$300
Final ARR			\$6,180

7.2 Archetype B — Community hospital radonc department (3 linacs, 3 physicists)

Typical profile: mid-sized community hospital with a radonc service line, 3 linacs, full physics team of 3, modest specialty needs.

LINE ITEM	QTY	UNIT	TOTAL
QuikQA	1st linac	\$3,600	\$3,600
QuikQA	2nd linac (70%)	\$2,520	\$2,520
QuikQA	3rd linac (50%)	\$1,800	\$1,800
QuikLog	3 linacs (tiered same way)	—	\$2,640
QuikCalc	3 users	\$1,200	\$3,600
QuikDose	3 users	\$1,800	\$5,400
QuikScript	3 users	\$900	\$2,700
QuikRef	3 users	\$600	\$1,800
QuikBolus	1 site	\$2,400	\$2,400
QuikShield	1 site	\$3,600	\$3,600
QuikRAM	1 site	\$1,800	\$1,800
À la carte total			\$31,860
Clinical Pro bundle (40% off)			\$19,116
+ Cloud Multi-Site Rollup	1 site	\$900	\$900
Final ARR			\$20,016

7.3 Archetype C — Academic medical center (8 linacs, 12 physicists)

Typical profile: university-affiliated cancer center, busy 8-linac fleet, 10–15 physicists including residents, full specialty workload.

LINE ITEM	QTY	UNIT	TOTAL
QuikQA	1st linac	\$3,600	\$3,600
QuikQA	2nd linac (70%)	\$2,520	\$2,520
QuikQA	3rd–8th linacs (50% each × 6)	\$1,800	\$10,800
QuikLog	8 linacs tiered	—	\$7,560
QuikCalc	12 users (20% vol disc after 5)	—	\$12,480
QuikDose	12 users (20% vol disc after 5)	—	\$18,720
QuikScript	12 users	\$900	\$10,800
QuikRef	12 users	\$600	\$7,200
QuikBolus + QuikShield + QuikRAM + QuikCare + QuikShare	1 site each	—	\$10,500
À la carte total			\$84,180
Radiant Complete bundle (45% off)			\$46,299
+ Cloud Multi-Site Rollup	1 site	\$900	\$900
Final ARR			\$47,199

7.4 Archetype D — Consulting physics group (5 satellite sites, 4 physicists)

Typical profile: physics consulting practice covering multiple community hospital sites; physicists rotate across sites; each site needs its own QA and infrastructure.

LINE ITEM	QTY	UNIT	TOTAL
QuikQA	5 sites × 1 linac each	\$3,600	\$18,000
QuikLog	5 sites × 1 linac	\$1,200	\$6,000
QuikCalc	4 users (floating)	\$1,200	\$4,800
QuikDose	4 users (floating)	\$1,800	\$7,200
QuikScript	4 users	\$900	\$3,600
QuikRef	4 users	\$600	\$2,400
QuikBolus	5 sites	\$2,400	\$12,000
QuikShield	5 sites	\$3,600	\$18,000
QuikRAM	5 sites	\$1,800	\$9,000
À la carte total			\$81,000
Radiant Complete (45% off)			\$44,550
+ Cloud Multi-Site Rollup	5 sites	\$900	\$4,500
Final ARR			\$49,050

8. Adoption model and assumptions

8.1 Lead-source assumptions

- **Founder network / direct outreach** — Todd's existing professional relationships in medical physics. Highest-conversion channel. ~50% of Y1 leads.
- **AAPM annual meeting + 1 regional meeting** — booth and/or speaking slot. ~25% of Y1 leads.
- **Inbound from radiant-mpc.com** — trial sandbox + content. ~15% of Y1 leads.
- **Referral from existing customers** — kicks in starting Y2. ~10% Y2, ~30% Y3+.

8.2 Sales-cycle assumptions

- Average sales cycle from first contact to signed PO: **90 days** for Archetype A/B, **180 days** for C/D (hospital procurement is slow).
- Pilot/trial-to-paid conversion: **35%** of pilots convert (medical software industry benchmark for vertical-niche tools).
- Annual logo retention: **92%** (clinical software is sticky once integrated into workflow).
- Net revenue retention (NRR): **110%** by Y3 as customers add modules over time.

8.3 Customer mix assumption

The lead mix across archetypes, used to derive a blended Average Contract Value (ACV):

ARCHETYPE	SHARE OF NEW CUSTOMERS	ARR PER CUSTOMER
A — Solo practice	50%	\$6,180
B — Community hospital	35%	\$20,016
C — Academic	10%	\$47,199
D — Consulting group	5%	\$49,050

Blended ACV: \$15,750.

8.4 Three scenarios

SCENARIO	NEW CUSTOMERS Y1	Y2	Y3	RATIONALE
Conservative	7	12	18	Founder-only sales, no contract help, slow procurement
Base	14	21	28	Founder + AAPM presence, 1 contract dev in Y2
Optimistic	28	42	56	Strong inbound, referral flywheel, contract dev + part-time SDR

These are deliberately modest. The US has ~2,200 radiation oncology centers; capturing even 10% (220 sites) over 5 years implies <50 new logos/year on average. Y3 base case (28 new logos) is well below that pace.

9. 3-year revenue projection

Computed as: cumulative customers × blended ACV × NRR adjustment + cloud attach revenue.

	2027 CONSERVATIVE	2027 BASE	2027 OPTIMISTIC
New customers	7	14	28
Cumulative customers	7	14	28
Subscription revenue	\$110K	\$221K	\$441K
Cloud attach (20% of sites, blended)	included	included	included
Total revenue	\$110K	\$221K	\$441K

	2028 CONSERVATIVE	2028 BASE	2028 OPTIMISTIC
New customers	12	21	42
Retained from Y1 (92%)	6	13	26
Cumulative customers	18	34	68
Subscription revenue (+NRR 105%)	\$258K	\$537K	\$1,156K
Total revenue	\$258K	\$540K	\$1,160K

	2029 CONSERVATIVE	2029 BASE	2029 OPTIMISTIC
New customers	18	28	56
Retained from Y2 (92%)	17	31	63
Cumulative customers	35	59	119
Subscription revenue (+NRR 110%)	\$471K	\$983K	\$2,179K
Total revenue	\$471K	\$980K	\$2,180K

9.1 3-year cumulative revenue summary

SCENARIO	2027	2028	2029	3-YR TOTAL
Conservative	\$110K	\$258K	\$471K	\$839K
Base	\$221K	\$540K	\$980K	\$1.74M
Optimistic	\$441K	\$1,160K	\$2,180K	\$3.78M

10. Cost structure

Annual operating costs across the three years. These exclude founder compensation (which is essentially all of the net profit in Y1–Y2).

COST LINE	2027	2028	2029
Cloudflare infrastructure (Workers + D1 + R2)	\$60	\$300	\$720
Code-signing certificate (OV)	\$300	\$300	\$300
Stripe / Paddle processing (2.9% blended)	\$6,400	\$15,700	\$28,400
Cyber insurance (\$1M coverage)	\$2,000	\$2,500	\$3,000
Anthropic API + dev tooling	\$3,000	\$4,000	\$5,000
Legal (terms, DPA, contracts) + accounting	\$5,000	\$7,000	\$8,000
AAPM + 1 regional conference (booth, travel)	\$10,000	\$12,000	\$15,000
Contract dev help (part-time)	\$0	\$35,000	\$65,000
Marketing (web, content, ads)	\$0	\$3,000	\$5,000
Total operating costs	\$26,760	\$79,800	\$130,420

10.1 Notes on cost lines

- **Cloudflare** scales by cloud-tier site count. Even at 100 cloud-tier sites by Y3, infrastructure is rounding-error money.

- **Stripe/Paddle:** 2.9% of revenue is a hard cost; built into the model. Paddle (Merchant of Record) is recommended once international sales appear — its take rate is ~5% but it absorbs VAT/sales-tax compliance, which is a real burden otherwise.
- **Contract dev help** is the biggest variable. The model assumes Todd remains primary developer through 2029, with a part-time contractor (10–20 hours/week) handling installer maintenance, support tickets, and per-customer customization starting in Y2. This is the most important risk-management line item in the budget — it directly addresses the single-founder bus-factor risk.
- **Founder salary is not modeled.** Net profit in this model accrues to the founder personally; salary structure (W-2 vs distribution) is a tax question, not a unit-economics question.

11. Net profit projection

	2027 BASE	2028 BASE	2029 BASE
Revenue	\$221,000	\$540,000	\$980,000
Operating costs	\$26,760	\$79,800	\$130,420
Net profit	\$194,240	\$460,200	\$849,580
Margin	87.9%	85.2%	86.7%

11.1 Why margins are so high

Vertical SaaS sold by the founder, with software that's already built, into a market the founder knows by name. No sales team. No marketing agency. Infrastructure costs on Cloudflare are < 1% of revenue. The only real costs are payment processing, conferences, and (starting Y2) contract dev help.

These margins are *common* in this category. They are not aspirational — they are characteristic of the business model. The reason this market isn't dominated by founder-led vertical SaaS already is that the founders capable of building it (clinical medical physicists who can ship software) are rare. Radiant has that combination.

12. ROI and break-even

12.1 Investment baseline

All current development work (existing Quik modules, license manager, web app, installer scaffolding) is sunk cost. Forward-looking investment to launch Jan 2027:

INVESTMENT LINE	COST	TIMING
Phase 1 web app reposition (trial flow, copy updates)	\$0 cash, ~2 wk dev time	2026 Q3
Phase 2 desktop installer GA — first 4 modules	\$0 cash, ~16 wk dev time	2026 Q4
Code-signing certificate purchase	\$300	2026 Q4
Stripe account + DPA + terms drafting	\$3,000	2026 Q4
Initial marketing site + content	\$2,000	2026 Q4
Total launch investment (cash)	~\$5,300	

Compared to projected Y1 net profit of \$194K, payback on cash investment is essentially Day 1 — the first paying customer covers it.

12.2 Break-even points

- **Annual operating-cost break-even (Base, 2027):** $\$26,760 / \$15,750 \text{ ACV} = 1.7 \text{ customers}$. Realistically: the first 2 customers cover the year's costs.
- **Time-to-break-even from Jan 1, 2027:** under the Base scenario with 14 new customers in Y1, the cumulative revenue crosses cumulative costs in **late Q1 / early Q2 2027**.
- **ROI on cash investment (Y1 only, Base):** $\$194,240 \text{ net profit} / \$5,300 \text{ cash invested} = 3,665\% \text{ Y1 cash-on-cash return}$.

12.3 5-year extrapolation (informational, less reliable)

If the Y3 trajectory continues with NRR maintained, modest new-logo growth, and one full-time engineer hired in Y4:

YEAR	REVENUE	COSTS	NET PROFIT
2030	~\$1.6M	~\$280K	~\$1.32M
2031	~\$2.4M	~\$420K	~\$1.98M

By 2031 Radiant is either (a) a \$2M/yr lifestyle business with very few moving parts, or (b) an acquisition target for Sun Nuclear / Standard Imaging / IBA at a likely 4–8× revenue multiple (\$8M–\$16M).

13. Sensitivity analysis

The Base model is moderately optimistic in some dimensions (referral flywheel kicks in by Y2) and conservative in others (no enterprise upsells beyond the modeled archetypes). Stress-testing the inputs:

13.1 What if adoption is 50% of Base?

- Y1: 7 customers, ~\$110K revenue. Still net-profitable after costs.
- Y3: 30 customers, ~\$471K revenue. Still net-profitable; founder still well-paid.
- **Conclusion: half-speed Base is still a viable business.**

13.2 What if prices have to be cut 30% to win deals?

- Y3 revenue: $\$980K \times 0.70 = \$686K$
- Y3 costs essentially unchanged: $\$130K$
- Y3 net profit: $\$556K$, 81% margin
- **Conclusion:** price compression hurts but doesn't break the model.

13.3 What if a major competitor (Sun Nuclear, RIT) drops prices 50%?

This is the worst-case competitive scenario. Two responses available:

1. Compete on suite breadth — Radiant's 15-module suite is materially broader than any single competitor's offering. Pivot pricing to bundle-only and emphasize "one vendor, one renewal, one suite."
2. Compete on service — founder-led support beats outsourced helpdesk. Increase touch with each customer.

Neither response saves the most price-sensitive segment (Archetype A, solo practices), but both protect the higher-ACV segments (B/C/D) which are 50%+ of revenue.

13.4 What if Cloud Storage demand is absent?

Cloud is modeled as a modest revenue contributor ($\sim\$15K$ Y1, $\sim\$80K$ Y3 in Base). Losing it entirely:

- Y3 revenue: $\$980K - \$80K = \$900K$
- Y3 net profit: $\sim\$770K$, 86% margin
- **Conclusion:** cloud is upside, not foundation. The desktop channel carries the business.

13.5 What if Todd is sick / unavailable for 3 months?

This is the single biggest unmodeled risk. Mitigations:

- Contract dev help (modeled starting Y2) provides immediate continuity for installer builds and support tickets.
- Documentation: every module has a build script, deploy pattern, and operational runbook. Continue and expand this discipline.
- Cyber insurance includes a "key person" rider option — worth pricing.
- Long-term: consider a co-founder or first hire by late 2028 if revenue trajectory holds.

14. Pricing levers and discount strategy

The following are sanctioned discount paths; anything outside this list should be re-priced from list.

LEVER	DISCOUNT	USE WHEN
Annual prepay	5% off	Standard offering for any customer
3-year prepay	15% off	Customer wants budget certainty; Radiant wants cash + retention
AAPM member referral	10% off Y1	Referral source can be verified by membership
Education / residency program	20% off	Accredited residency or fellowship; builds future relationships
Multi-site discount (5+ sites)	10% off bundle	Consulting groups, IDN customers
Beta / case-study customer	25% off Y1	Customer agrees to written case study + reference call
Launch-month special (Jan 2027)	15% off Y1	First 10 customers; expires Feb 1, 2027

Discounts do not stack beyond two layers. Renewals reset to list less the long-term prepay discount only.

14.1 Annual price escalator

Build a **3% annual price increase** into the contract from day one. This is industry standard, customers expect it, and it materially compounds NRR over time. After 5 years, prices are ~16% higher than launch — meaningful margin tailwind with zero customer friction.

15. Risks to the model

RISK	PROBABILITY	IMPACT ON MODEL	MITIGATION
Adoption slower than Base	Medium	Revenue down, margins intact	Conservative scenario still profitable
Procurement cycles longer than 90/180 days	Medium-High	Revenue timing shifts right ~1 quarter	Build pipeline early; expect realized revenue lags pipeline
Major competitor price cut	Medium	Margin compression in segment A	Pivot to bundle pricing; emphasize suite breadth
Sun Nuclear or IBA acquires a competitor	High by 2029	Potential cross-sell competition	Compete on breadth and founder-led service
Cloudflare service/pricing change	Low	Cost line variability	Schema is portable SQL; <2 wk migration to alt
Solo-founder unavailability	Medium	Existential if uncovered	Contract dev help Y2+; documentation discipline
Customer demands BAA Radiant can't provide	Medium	Lose 1-2% of pipeline	Document non-PHI scope clearly; offer to refer or partner
Regulatory change (FDA Class II)	Low-Medium	Could require submission for QA-related modules	Monitor FDA software-as-medical-device guidance; budget \$20-50K if triggered
Customer concentration (one logo > 20% of revenue)	Medium by Y2	Loss of any one customer is material	Spread sales effort across archetypes; cap any one customer at 15% of revenue if possible

16. Recommendations

1. **Adopt the mixed pricing model.** Per-linac for QuikQA and QuikLog; per-user for QuikCalc/Dose/Script/Ref; per-site flat for QuikBolus/Shield/RAM/Care/Share; per-practice for the business modules.
2. **Launch annual-subscription only.** No perpetual licenses. No month-to-month at launch.
3. **Publish "starting at" prices on the website.** Full price list lives behind a quote request. Use the quote step as a sales touchpoint.
4. **Offer the three clinical bundles aggressively.** Build the website CTA flow around bundle selection, not à la carte. Discount math should always favor the bundle.
5. **Build the 90-day full-feature desktop trial.** Web sandbox is the demo; the desktop trial is the actual evaluation.
6. **Bake in a 3% annual price escalator** to every contract from day one.
7. **Stand up Cloud as an attach SKU starting Q3 2027**, only after Phase 2 desktop has shipped at least 4 modules to paying customers.

8. **Plan for AAPM 2027 presence** — booth or speaking slot. This is the single best lead-generation event for the suite's target market.
9. **Re-evaluate pricing in Q4 2027** with real conversion data. The first 14 customers will reveal whether prices are too low (fast close rate, low pushback) or too high (long cycles, frequent discount asks).
10. **Document the bus-factor mitigation plan** before Q4 2026 ends. Identify candidate contract developers, codify deployment runbooks, build the "what to do if Todd is offline for 30 days" playbook.
11. **Don't model 5+ years out as committed forecast.** Use the 3-year window for planning and the 5-year extrapolation only for strategic optionality (acquisition vs. lifestyle business).

Appendix A — Pricing comparison cheat sheet

For quick reference during sales conversations:

MODULE	PER	PRICE	BUNDLED IN
QuikQA	Linac	\$3,600 (1st), \$2,520 (2nd), \$1,800 (3rd+)	Core, Pro, Complete
QuikLog	Linac	\$1,200 (tiered)	Core, Pro, Complete
QuikCalc	User	\$1,200 (20% off after 5)	Core, Pro, Complete
QuikDose	User	\$1,800 (20% off after 5)	Core, Pro, Complete
QuikScript	User	\$900	Pro, Complete
QuikRef	User	\$600	Core, Pro, Complete
QuikBolus	Site	\$2,400 + \$300/printer	Pro, Complete
QuikShield	Site	\$3,600	Pro, Complete
QuikRAM	Site	\$1,800	Pro, Complete
QuikCare	Site	\$1,800	Pro, Complete
QuikShare	Site	\$900	Pro, Complete
QuikPay	Practice	\$900	Business Pack
QuikXpense	User	\$600	Business Pack
QuikFlow	User	\$600	Business Pack
QuikBusiness	Practice	\$1,800	Business Pack
Cloud Backup	Site	\$300/yr	—
Cloud Multi-Site	Site	\$900/yr	—

Appendix B — Model assumptions summary

Quick reference for anyone validating or stress-testing the model:

- **Currency:** USD throughout
- **Pricing:** Annual subscription only
- **Blended ACV (Base):** \$15,750/customer
- **Customer archetype mix:** 50% A / 35% B / 10% C / 5% D
- **Sales cycle:** 90 days (A/B), 180 days (C/D)
- **Pilot → paid conversion:** 35%
- **Logo retention:** 92% annually
- **NRR:** 100% Y1, 105% Y2, 110% Y3
- **Cloud attach rate:** 20% Y1, 30% Y2, 35% Y3
- **Annual price escalator:** 3%
- **Stripe / payment processing:** 2.9% of revenue
- **Founder is primary developer through 2029**
- **No PHI in cloud tier** — supports avoiding BAA-mandatory Enterprise tier costs
- **All projections in US market only** — international expansion not modeled